MISSION BAY NEWS

Mobile Home Community

1971 *February 2025*

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Monthly Newsletter

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PARK SECURITY

If you see anyone who looks like they do not belong in the park, especially at an odd hour, CALL 911 FIRST

The San Leandro Police has advised us to leave our front and back porch light on all night.
NON-EMERGENCY number for the police dept. is (510) 577-2740

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Mission Bay News Crew

Many thanks to our News Crew who organize and deliver our monthly magazine! You are appreciated!

Alice Leung	Coordinator
Brenda Gianni	Activity Calendar
Vince Crabill	Distribution
Michael Garris	1 thru 33 | 501 thru 541
James Gamble	50 thru 72 | 150 thru 197
Vince Crabill	101 thru 145
Marta Barnes	201 thru 239
Rosita Gee	250 thru 289
Nancy Garrett	301 thru 338
Irene Zhong	350 thru 387
Darrin McWilliams	401 thru 440
Angie Chan	450 thru 491

ACTIVITY CALENDAR

FEBRUARY 2025



SUN.	MON.	TUES.	WED.	THURS.	FRI.	SAT.
						1 TAI CHI 10 – 11
2 	3 EXERCISE CLASS 9:30-10:30 LINE DANCING 3 - 4	4 CARD GAME 12:30 – 4	5 TAI CHI 10:30 – 11 MEXICAN TRAIN 1:30-3:30 POKER GAME 12-3:30 EXERCISE CLASS 9:30-10:30	6 TAI CHI 10:30 – 11 CARD GAME 12 – 4:30 (Crafts Rm) CARD GAME 12:30 – 4	7 POKEENO 5 – 7:30 EXERCISE CLASS 9:30-10:30	8 BIG TENT/ MBRA MTG 9:30 – 12:30
9 	10 EXERCISE CLASS 9:30-10:30 LINE DANCING 3 – 4	11 CARD GAME 12:30 – 4	12 TAI CHI 10:30 – 11 MEXICAN TRAIN 1:30-3:30 POKER GAME 12-3:30 EXERCISE CLASS 9:30-10:30	13 TAI CHI 10-10:30 BOOK CLUB 1 pm (Crafts Rm) CARD GAME 12:30 – 4	14 POKEENO 5 – 7:30 BIBLE STUDY 6:30 pm (Crafts Rm) EXERCISE CLASS 9:30-10:30 RECYCLE 	15 TAI CHI 10 – 11
16	17 EXERCISE CLASS 9:30-10:30 LINE DANCING 3 – 4 	18 CARD GAME 12:30 – 4	19 TAI CHI 10:30 – 11 MEXICAN TRAIN 1:30-3:30 POKER GAME 12-3:30 EXERCISE CLASS 9:30-10:30	20 TAI CHI 10-10:30 CARD GAME 12 – 4:30 (Crafts Rm) CARD GAME 12:30 – 4	21 POKEENO 5 – 7:30 BIBLE STUDY 6:30 pm (Crafts Rm) EXERCISE CLASS 9:30-10:30	22 TAI CHI 10 – 11 M & W POT LUCK 4-6
23	24 EXERCISE CLASS 9:30-10:30 LINE DANCING 3 - 4	25 CARD GAME 12:30 – 4	26 TAI CHI 10:30 – 11 MEXICAN TRAIN 1:30-3:30 POKER GAME 12-3:30 EXERCISE CLASS 9:30-10:30	27 TAI CHI 10-10:30 CARD GAME 12 – 4:30 (Crafts Rm)	28 POKEENO 5 – 7:30 RECYCLE EXERCISE CLASS 9:30-10:30	

BIRTHDAYS & ANNIVERSARIES

FEBRUARY
Birthdays

Barbara Betz	3
Nidia Sanchez	6
Sharron Kahoalii	7
Evelyn Lucero	9
Candance Griggs	9
Louise Pocengal	9
Kathy Enos	10
Ponchita Sheppard	14
Tom Maitre	17
Marta Barnes	21
Michelle Richards	25
Lynda Ronguillo	27
Donna Garfinkle	28
Kara Music	28

FEBRUARY
Anniversaries

Fidel & Nidia Sanchez 1





G S M O L
CHAPTER 518
J O I N U S

The Golden State Manufactured-Home Owners League (GSMOL) is a non-profit organization formed almost 60 years ago to protect the home values and quality of life of Mobile Home owners. In addition to working with the State of California and local legislatures, GSMOL works daily at the State level lobbying for legislation that will benefit Mobile Home owners. Through our Emergency Legal Fund (ELF), GSMOL pursues violations of the Mobilehome Residency Law (MRL798…) which protects Mobile Home owners.

To help GSMOL continue to provide these and other services,

we invite you to join today.

MEMBERSHIP APPLICATION

GOLDEN STATE MANUFACTURED-HOME OWNERS LEAGUE, INC. 800-888-1727 714-826-4071 FAX : 714-994-9637

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- ☐ ONE-YEAR *ASSOCIATE* MEMBERSHIP for $25

(Associate members do not own manufactured homes. They do not have voting rights and cannot hold office in GSMOL, but do receive The Californian magazine)

(DUES ARE NON-REFUNDABLE)

Comments (For Office Use)

First Name | Initial | Last Name

Spouse/Second Occupant

Park Name

Street Address | Space Number

City | State | Zip Code

Daytime Phone Number | Alternate Phone Number

Email Address

Signature | Membership Recruiter (if applicable)

☐ New Member
☐ Renewing Member

GSMOL Chapter # _____

Check # _____ / CASH

You can also contribute to any of the following GSMOL dedicated funds

Legal Defense Fund $____

Disaster Relief Fund $____

Political Action Committee (PAC) $____

FILL OUT AND RETURN THIS FORM ALONG WITH YOUR CHECK TO GSMOL, 14802 BEACH BLVD., LA MIRADA, CA 90638

DETACH AND KEEP FOR YOUR RECORDS Thank you! Check# ____ Amount ____ Date ____

GSMOL

Form 100
Rev 04/16

Please deposit membership dues in the GSMOL box located in the clubhouse.

Thank You!

VIEU NEWS

BY PETER CLEAVELAND

Can We Survive Climate Change?

The Earth is getting hotter

This means that the sea level will continue to rise, and coastal areas will increasingly erode and/or flood. Large amounts of farmland around the world will become too dry for crops, and large areas of the planet will become too hot for human habitation. Many plant and animal species will become extinct. If we can't stop it, how can individuals and nations adapt and survive? Let's look at the hazards and some approaches to them.

Rising sea levels

In the short run — 100 years or so — it will be possible to protect some coastal areas with new and higher sea walls and other barriers. One fix for seaside houses is to elevate them, allowing storm waters to flow underneath without too much damage, but this is only a temporary patch. We can also restore coastal wetlands, which can help reduce flooding damage due to storms.

In the long run, of course, some island nations will simply disappear. Other low-lying nations, like Bangladesh, will lose much of their areas. For the rest, the only answer is to move to higher ground.

Heat

Large areas of the planet will, in the next few centuries, become simply too hot for humans to survive. These include much of the Amazon Basin, North Africa, the Levant, India and South Asia, and the tropics in general. This will lead to mass migrations, with the resulting political upheavals. In the mean time, there are things we can do. We can plant trees — lots more trees. In cities, we can paint roofs white to make them reflect more sunlight.

Next month we will go further into this.

VIEU新闻
我们能否在气候变化中生存？

地球正在变得越来越热。

这意味着海平面将继续上升，沿海地区将面临日益严重的侵蚀和/或洪水问题。世界范围内的大量农田将因为干旱而无法种植作物，而地球的许多地区将变得过于炎热，不适宜人类居住。许多动植物物种将面临灭绝。如果我们无法阻止这一趋势，个人和国家应该如何适应并生存？让我们一起探讨气候变化带来的危害以及可能的应对措施。

海平面上升

短期内——大约100年左右——我们可以通过建造新的、更高的海堤和其他屏障来保护一些沿海地区。对于沿海的房屋，可以将房屋抬高，让风暴潮水从房屋下流过，从而减少损害，但这仅是暂时的解决方案。我们还可以恢复沿海湿地，这有助于减轻风暴造成的洪涝损失。

从长远来看，一些岛国将彻底消失。而像孟加拉国这样的低洼国家也会失去大部分土地。对于其余国家，唯一的解决办法是迁移到地势更高的地方。

高温

在未来几个世纪内，地球上大面积地区将变得过于炎热，不适合人类生存。这些地区包括亚马逊流域的大部分地区、北非、黎凡特、印度及南亚，以及热带地区。这将导致大规模的人口迁徙，从而引发政治动荡。

与此同时，我们还是可以采取一些措施。例如，我们可以种植更多的树木——大量的树木。在城市中，我们可以将屋顶涂成白色，以反射更多的阳光。

下个月，我们将进一步探讨这一问题。

February Men and Women's Club News

YOU ARE INVITED TO

POTLUCK DINNER

22 FEB

BRING YOUR FAVOURITE DISH TO SHARE: MAIN DISH, SIDE, SALAD OR DESSERT

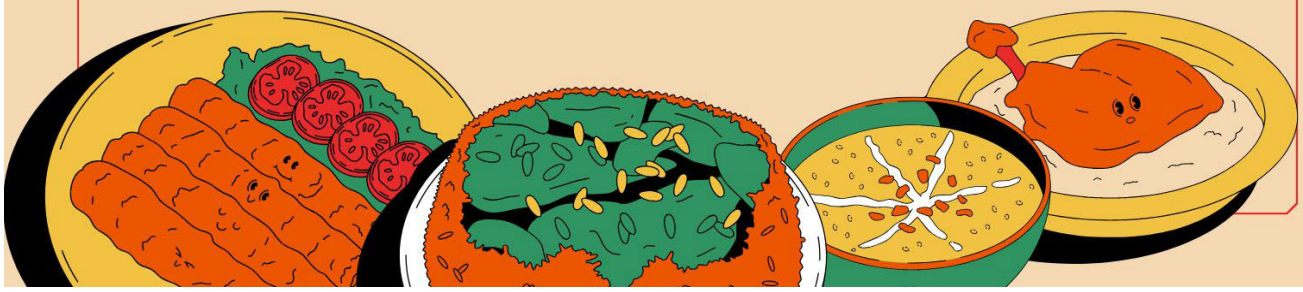
SATURDAY, AT 4 - 6 PM
CLUBHOUSE

NO COST
50/25/25 RAFFLE



SIGN UP SHEET WILL BE IN CLUBHOUSE BY FEBRUARY 9TH

JOIN YOUR FRIENDS AND NEIGHBORS FOR A FUN EVENING OF FOOD AND FELLOWSHIP!



WORD SEARCH

Be My Valentine

N	L	G	I	E	G	I	O	G	B	A	T	I	N
O	R	D	E	V	O	T	I	O	N	H	I	E	A
I	E	I	G	F	E	E	O	S	S	A	L	O	T
T	R	Y	N	O	T	I	N	S	O	O	G	P	N
C	A	F	R	N	D	S	E	B	G	N	E	C	E
E	N	N	S	D	G	N	O	I	I	C	E	D	S
F	E	O	V	N	R	R	A	R	R	E	D	C	F
F	A	G	I	E	S	S	A	A	L	A	V	H	A
A	R	T	D	S	F	C	D	G	F	N	G	E	A
F	F	N	E	S	S	N	E	A	R	S	O	R	R
A	E	T	N	E	O	A	E	P	I	L	A	I	D
T	N	S	S	S	N	N	P	E	E	E	T	S	O
S	I	C	I	N	N	G	S	R	N	I	R	H	I
B	O	Y	F	R	I	E	N	D	D	A	H	F	G

CARING
AGAPE
BOYFRIEND
AFFECTION
TENDERNESS
FONDNESS
PASSION
CHERISH
DEVOTION
GIRLFRIEND

Relationship Status: Single — Dating Among Grown-Ups

by Vicki Gelfeld, AARP Research



Are single GenXers, Boomers, and grandparents ready to mingle? Many seem to be at least open to it, according to a new AARP survey.

When it comes to dating, most single GenXers (78%), Boomers (62%), and grandparents (56%) are either in the game or open to the experience, the survey found.

In spite of a general openness to dating, the majority of single GenXers (70%), Boomers (82%), and grandparents (78%) are not currently doing so. Why? Many say they are dissuaded by the casual attitude potential partners take and the challenges in finding a compatible person.

For those willing to put themselves out there to date, most say it's often more about finding companionship than getting married or remarried. Among single grandparents, women (45%) are more likely than men (25%) to be looking for companionship, while more men (34%) tend to want a partner (versus 17% of women). For single GenXers, 34 percent do have marriage on their minds compared to 15 percent of Boomers, according to AARP.

The national survey of single grown-ups over the age of 38 revealed that most prefer to have friends introduce them to new people (55% of GenXers, 60% of Boomers, and 63% of grandparents). Although 37 percent of single grandparents say family members are the best resource, only about half of those who are open to having family introduce them to new people have actually done so (46%), and they are more willing to have their children play matchmaker (30%) than their grandchildren (18%).

Although the dating landscape has changed significantly in recent years, AARP reports most single grandparents are fairly traditional in their approach, even though they are not looking for marriage. Most prefer an offline course of action. GenXers are more apt to rely on technology, however, with 23 percent believing online dating sites and 16 percent believing social media are the best matchmakers, compared to 15 percent of Boomers regarding dating sites and 6 percent

for social media. When communicating with a new romantic interest, GenXers are twice as likely as Boomers to prefer texting them.

Most grown-ups said the best first dates would be over coffee, lunch, or dinner rather than going to a happy hour or doing an activity together. GenXers are much more likely than Boomers to prefer a dinner date, though, as well as to like the idea of a sport or unique experience to get to know someone new.

More than half of single grandparents believe a man should pick up the tab for the first date, although men hold that view in much stronger numbers (86%) than women (53%). While 49 percent of Boomer women and 42 percent of GenX women would say no to a kiss on the first date, 20 percent of Boomer men and 16 percent of GenX men report the same. As for follow up after the date, GenX women are also taking the reins there, with 72 percent saying they would contact a man after a great date, while 60 percent of Boomer women would do so.

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9 Ways to Look Stylish and Self-Assured on a First Date

By Lois Joy Johnson



So, you're ready to dive into the dating pool again at 50-plus. That first date might be a dating-site pick, a family fix up or a “let's grab a coffee” from a high school ex you friended on Facebook. Whatever the source, it's an audition. At the very least you'll get some practice and rediscover feelings and parts of yourself that may have been hidden (or lost) for years. At most you gain a new dinner companion or a blazing romance. And as Ariana Grande sings, if the first date doesn’t work, you can always bow out with a “thank you, next!” Here are nine empowering beauty and style tips to get you going.

1. Loosen up your hair

Hair is still a big part of physical attraction, but don't let thinning hair get in the way of sex appeal. Sensual hair moves. A swingy, glossy bob can be alluring — so can a cropped pixie with a just-ran-my-hands-through-it look. Count on any of the following for the right effect: beachy waves, springy curls, shaggy layers, a side part or flirty full bangs. This is not the time to pull your hair back in a tight ponytail or wear a hat. Trust three-times-married, beauty-editor me on this one.

2. Be high visibility from the waist up

You'll hopefully be meeting up in a public place like a coffee shop or cafe. Wear a scarf top or jacket that stands out in a crowd. A color that's energetic like a red is easy to spot and sends a friendly, approachable message. And text it ahead: “I'm by the bar wearing a big orange scarf."

3. Show off your best, keep the rest to yourself

Let's be honest, a waist adds appealing body

definition. And if you have one, wear a belt to make that point. But if your torso isn't cooperating with crunches or your postmenopausal diet, focus on revealing your long elegant neck, your toned arms, spicy shoulders or long dashing legs. The extremities are always good eye-catching spots. Dig out those cold shoulder sweaters you bought last year, stack your bracelets, stick to V necks, or unbutton your shirt to just above the cleavage and wear leggings — leather ones are great even with a tunic sweater.

4. Don't look like you're trying too hard

Jeans work everywhere on everyone, so have your favorites on call. Dressing them up or down is easy. A silk blouse or a V-neck sweater with low heels or pointy flats looks feminine and polished; a simple white T-shirt or crisp white shirt with sleeves turned up and rolled with leather ankle boots looks classy and classic. White jeans with a black blouse or sweater and your black belt are always memorable. A long rock-star scarf casually wrapped or draped adds just enough style. Save the dresses, heels and leather pencil skirts for later developments.

5. Have some edge

Always wear a black bra and panties — even under a white tee. (This is for you only.) Then add something a little unexpected or wild — snakeskin booties, those leather leggings (see No. 3), a slogan tee under a fitted blazer, or a buttery leather biker jacket with a bow blouse for a provocative counterpoint. A hint of dark humor or the rebel in you is not something that should be hidden, and it can also



6. Look like you ... right now

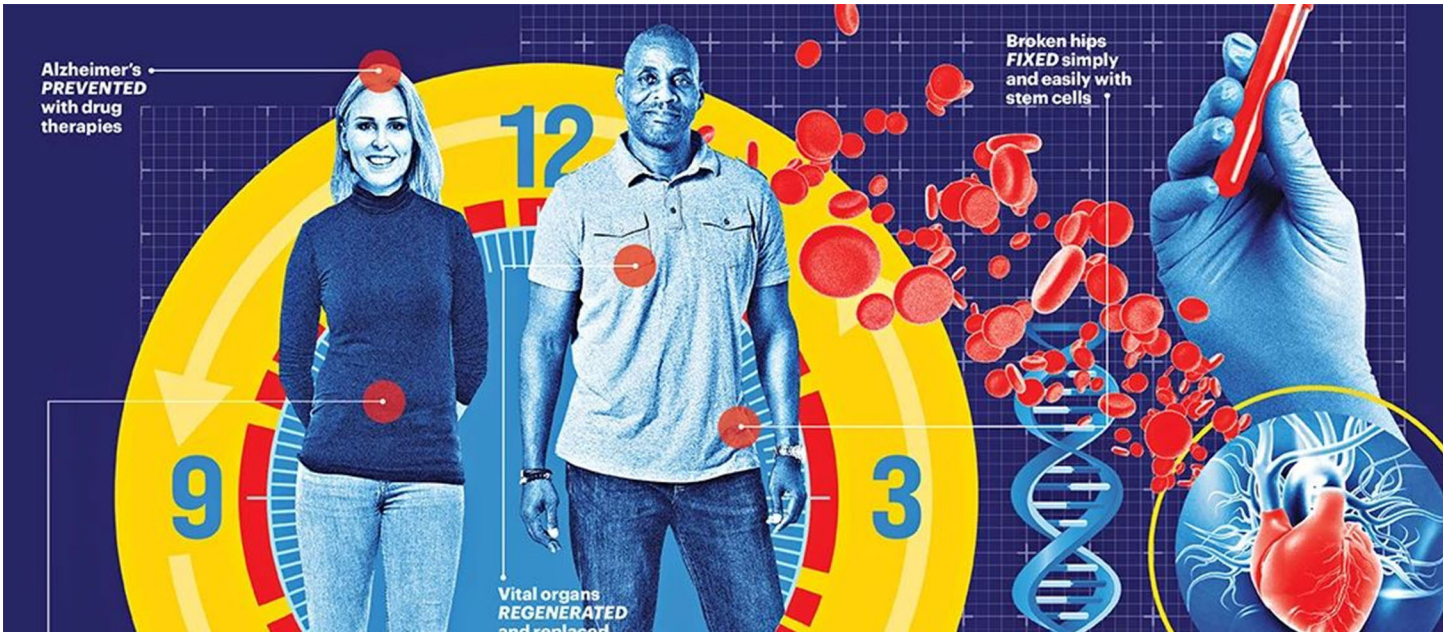
Listen: It's OK to use filters to tweak online photos — not to deceive but just to brighten skin, soften lines, clean up whites of your eyes and teeth. As a senior your weight, hair and looks do change, and these do happen: a few extra pounds here or there; new bangs or natural do instead of stick straight; short hair instead of long; or a hankering to wear flaming red lipstick. Don't apologize or explain. Just show up. A little narcissism is healthy. We're highly experienced, intelligent, informed, centered, accomplished, optimistic and attractive women. It's good to feel that power, and that includes the right to keep evolving the way we look.

7. Let your eyes speak with makeup

Listen well. A knowing glance, a wink and twinkle add a little mystery that's super attractive. Treat your peepers to a new stash of eye cream, liners, neutral shadows and mascara, and use them for a predate "pep talk." Blend a dab of eye cream with under-eye concealer for a wide-awake look; add a subtle, shimmery shadow in sand or peach on lids; use a dark gel pencil liner at the upper lash line and inner rim for discreet shaping. Apply eye makeup even under your fabulous statement glasses.

Medical Breakthroughs That Could Change the Way We Age

By Jeanne Dorin McDowell, AARP



Would you want to live to 200?

The question sounds hypothetical. But thanks to a handful of scientific breakthroughs that have emerged in just the past two years, it might not be.

In 2023, Tony Wyss-Coray and his team at Stanford University were able to calculate the rate of aging of 11 major organs using proteins in the blood known as biomarkers. And this past July, researchers in Sweden announced they'd found that a simple blood test could detect Alzheimer's disease with about 90 percent accuracy.

These discoveries have helped to create a new foundation upon which researchers can build, with the promise of detecting, treating and even halting emerging diseases—think heart disease, cognitive decline and many forms of cancer — before they have a chance to make us ill.

A mind-boggling tsunami of research is suddenly emerging from universities all over the world. In labs, old, frail mice that share blood with younger mice become healthier, stronger and live longer. Researchers believe this technology could one day be applied to humans. Such advances, which not too long ago were the domain of sci-fi novels and superhero movies, are now within sight.

Focusing on 'health span,' not lifespan

Today, the maximum human lifespan is estimated at somewhere between 115 and 120 years. (Frenchwoman Jeanne Calment, believed to be the oldest person who ever lived, died in 1997 at 122.) But researchers who study aging are no longer just focused on longevity. Instead, the end game is long life without many of the diseases that are associated with aging — not lifespan, but health span.

"We're not looking to have people live forever," says Thomas Rando, director of the UCLA Broad Stem Cell Research Center in Los Angeles. "We're looking to have them lead healthy lives for as long as they live. That's the dream."

In their quest to improve health span, more researchers are studying the field of "super agers" — people over 80 whose memory is at least as good as those in their 50s and 60s. What separates these late-life high achievers from the average population? And how can the rest of us catch up?

"On average, individuals experience cognitive decline with each successive decade from our 30s and 40s onward," says neuroscientist Emily Rogalski, director of the University of Chicago Healthy Aging & Alzheimer's Research Care (HAARC) Center, who first defined the term super ager. "Identifying key factors that allow for youthful memory holds promise for helping others extend their health span and avoid Alzheimer's and related dementias."

Indeed, the biggest risk factor for chronic diseases such as heart disease, Alzheimer's disease, type 2 diabetes, cancers, osteoarthritis — even hearing loss — is simply getting older. If we can slow the rate of aging, we can also delay, and perhaps prevent, the onset of disease, allowing people to live longer and healthier.

"We've identified some dials we

can tweak that allow us to change the rate of aging," says Eric Verdin, president of the Buck Institute for Research on Aging.

"The hypothesis is not fully proven yet, but the evidence is pretty strong," he says. "The aging field is transforming medicine."

This new way of looking at disease has triggered a tectonic shift in the science of aging, a field of research referred to by a different name: geroscience. The goal of geroscience is to extend physical health and cognition and make being a super ager the rule, rather than the exception.

When it comes to aging, 'we need a radical new approach'

Preserving the health of people in their 80s, 90s and beyond is a critical challenge facing the U.S. Over the next 30 years, the number of centenarians in this country is projected to quadruple to roughly 422,000 by 2054, according to the U.S. Census Bureau, while the population of people over 65 may reach 82 million in the next 25 years.

"At least half of these individuals over 65 will have two or more diseases; a quarter of them will have three or more diseases by age 70," says biochemist Laura Niedernhofer, director of the Institute on the Biology of Aging & Metabolism at the University of Minnesota. "We need a radical new approach."

Some researchers believe the era of that radical new approach has already dawned.

In the not-too-distant future, your doctor will be able to use antiaging supplements and drugs to "treat" aging overall, delaying the onset of age-related diseases. Advances in screening technology, together with targeted therapies, may usher in "a new era in the detection and treatment of cancer," says Ronald DePinho, cancer biologist and the Harry Graves Burkhart III distinguished university chair and past president of the University of Texas MD Anderson Cancer Center.

In 10 years, for example, if you or a loved one is diagnosed with pancreatic cancer — which today has a five-year survival rate of just 13 percent — a combination of targeted therapy, immunotherapy drugs activating the immune system, and a personalized mRNA vaccine (which triggers a cancer-specific immune response) could lead to "doubling or tripling five-year survival," DePinho says.

In 20 years, minimally invasive image-guided surgeries, followed by personalized vaccines after surgery, will help prevent recurrence of early-stage cancers.

In the coming decades, a hip fracture — which currently results in death in 21 percent of people 60 and older within a year after a fall — will be transformed from a potential tragedy into a temporary setback: Stem cell therapy, which may be delivered in a special infusion center, will allow older people to regrow bone mass, enabling them to return to full function and health, UCLA's Rando says.

Your annual checkup will most likely be a lot different as well. Beyond basics such as glucose levels and triglycerides, the checkup of the future may consist of testing thousands of biomarkers — molecules found in your blood and other body fluids or tissues that can reveal potential or emerging diseases before they pose a threat.

Although biomarker-guided therapies have been used to treat cancer for decades, scientists are identifying biomarkers of aging that will help predict dementia, liver disease, osteoporosis and other diseases, allowing for earlier and more accurate interventions before disease sets in.

Some scientists, such as Harvard geneticist and longevity researcher David Sinclair, believe that aging can even be reversed. Here's the theory: Day by day, year by year, our DNA replicates itself as we discard old cells and grow new ones. But like a copier running low on ink, the duplication gets less and less accurate, and genetic information is lost. That's what creates aging.

What would happen if we could prevent that information loss?

Last year, in a paper published in Cell, Sinclair and his team made the case that a backup copy of the genetic instructions stored on the body's "hard drive" could be rebooted, essentially reversing the damage done by aging. Though his experiments are primarily in the animal testing phase, Sinclair says he will start a trial next year to test this theory, with the specific goal of reversing blindness in humans.

Different body parts age at different rates

You need only look around at your circle of friends to realize that the passing of time affects everyone differently; one 70-year-old may look 60ish (hopefully that's you) while another looks fully their age and more. Each person has what's called a "biological age," which is the measure of the health of their tissues and organs. Your biological age may be lower or higher than the number of birthdays you've had.

"Everyone ages differently due to genetics, lived experiences and family history," says Miranda Orr, a neuroscientist specializing in Alzheimer's disease and brain aging at Wake Forest University School of Medicine and the Salisbury VA Medical Center in North Carolina. Even different parts of our bodies age at different rates, which is why an otherwise healthy person can be blindsided by an unexpected disease that targets one specific

organ — kidney failure, for example, or prostate cancer.

"We are a collection of organs, and not everyone's organs are aging at the same rate," says Wyss-Coray, the D.H. Chen distinguished professor of neurology and neurological sciences at Stanford University. In his groundbreaking study last year, Wyss-Coray not only demonstrated that researchers could use biomarkers to identify how each organ is aging, but was able to show the consequences of those findings: An organ that was aging faster carried a 15 to 50 percent higher risk for mortality in the following 15 years. One in 5 healthy adults 50 and older have at least one organ that is aging too fast, the study found.

Since identifying what they call the "hallmarks of aging" — the molecular, cellular and systemic processes that our bodies undergo as we grow older — geroscientists like Wyss-Coray can define the specific ways in which each individual ages. Once your unique hallmarks are established, doctors will be able to tailor drug regimens and combination therapies to treat your specific aging profile, rather than giving you the same type of treatment as everyone else.

And that opens up a wide world of opportunities to detect, treat and reverse disease.

The code to your specific aging profile is found within a series of these biomarkers — molecules within the body that offer clues to one's health. Triglycerides, A1C and prostate-specific antigens (PSA) are common examples of biomarkers. But researchers can now use blood, skin or saliva samples to analyze specific patterns in DNA, called methylation marks, that give a more detailed look at the biological age of various body parts.

If DNA from a certain area of the body shows signs of greater wear and tear, doctors may be able to administer treatments to give that area a helping hand — bringing its biological age back in line with the rest of the body before a disease can take hold.

"If you can monitor the relative age of an organ on an ongoing basis and see, for example, that the lung of a 60-year-old is aging like that of a 65-year-old, you can do an intervention to bring it back to its normal age," says Wyss-Coray, who predicts that the Food and Drug Administration (FDA) will approve tests for use in humans specifically designed to measure organ aging within the next five to 10 years.

Early detection may make Alzheimer's treatable at last

For nearly 7 million Americans who have Alzheimer's disease and millions of others whose family histories point to a strong proclivity for the disease, the challenges of diagnosis remain considerable.

"The problem is we don't understand the intrinsic mechanisms of the disease yet, so we don't have highly effective interventions, and we don't have enough knowledge about Alzheimer's to cure it," says Luigi Ferrucci, scientific director of the National Institute on Aging (NIA). But as with other parts of the body, early detection and intervention will probably be the key to effectively treating diseases of the brain. (The only two FDA-approved treatments currently available to slow progression of Alzheimer's, the anti-amyloid drugs lecanemab and donanemab, have to be administered in the early stages of the disease.)

Until now, the diagnosis of Alzheimer's has been made through PET scans and spinal taps — expensive procedures often inaccessible to people who live in rural areas or who lack insurance or money for pricey diagnostics. That's why the announcement this past summer by the group of researchers at Lund University in Sweden of the blood test that can accurately diagnose Alzheimer's in routine health care settings is a game changer.

Rhoda Au, a professor of anatomy and neurobiology at the Boston University Chobanian & Avedisian School of Medicine, is experimenting with voice-based digital markers as a diagnostic tool for Alzheimer's disease. Traditional cognitive testing for neurodegenerative illnesses can be time-consuming and inaccurate. Using voice-based digital markers, Au has found that people with cognitive impairment often have detectable acoustic features such as hesitation and stuttering that indicate disease much sooner than other diagnostic tools.

"The idea is to pick up that something is amiss early, then figure out why," Au says. "If you're experiencing a neurodegenerative process, it's not acute but gradual. The longer you can delay the progression or onset of symptoms, the longer you can delay significant consequences of this disease. The goal is to live to end of life cognitively intact."

As early diagnostic techniques improve, so do treatment options. Drugs that reduce inflammation in the brain are in clinical trials. The anti-amyloid drugs currently available only slow the disease by about 30 percent, says Howard Fillit, cofounder and chief science officer of the Alzheimer's Drug Discovery Foundation, "so we clearly need drugs with other mechanisms to be used in combination with anti-amyloid drugs to slow the disease down further." The goal is to slow the disease by 100 percent, he says. "Drugs reducing inflammation in Alzheimer's disease could achieve this goal, at least in part."

DePinho's team at MD Anderson recently identified a molecule that reduces age-related inflammation and improves brain and muscle function in mice. This could have enormous therapeutic implications for diseases such as Alzheimer's and Parkinson's, as well as heart disease and various forms of cancer.

Are medicines that 'cure' aging already on drugstore shelves?

Although there are no antiaging drugs approved by the FDA — in part because the FDA doesn't recognize aging as a disease — at least a dozen drugs that have been approved for other purposes are strong contenders for use as antiaging agents, says Nir Barzilai, president of the Academy for Health & Lifespan Research, whose research focuses on repurposing drugs to fight aging in new ways. These drugs target the hallmarks of aging and have protective mechanisms that work on the whole system to tamp down inflammation. Dozens of drugs are in clinical trials in humans.

Some of these medications, such as the diabetes drug metformin, the immunosuppressant rapamycin and ACE inhibitors that treat high blood pressure, have been around for decades. They're relatively inexpensive and have a long safety record. Researchers are starting to test these drugs and supplements in clinical trials in hopes that, if they work, the FDA will approve them as antiaging drugs.

Rapamycin, which has long been used to reduce organ rejection in transplant patients, is being studied for use on periodontal disease, a chronic inflammatory condition of the gums that affects many people over 65 and is associated with dementia and heart disease. And metformin has captured the interest of the FDA, which is advising on a study of metformin being launched by the American Federation for Aging Research (AFAR).

"We're making progress, but nothing is out there yet," Barzilai says. "We need to start using these drugs."

Another set of drugs in clinical testing targets senescent (or "zombie") cells. As we get older, the cells in our bodies lose their ability to divide and replicate — the key to life. These zombie cells accumulate in tissues throughout the body and release harmful substances. As the immune system responds to the zombie cells, it further fuels low-level, chronic inflammation that accelerates cellular damage, erodes muscle strength and weakens immune function — a process known as inflammaging.

Drugs are being tested that can target and eliminate zombie cells while sparing healthy cells; several have recently moved into early-stage human trials. A combination of the cancer drug dasatinib and quercetin, a naturally occurring nutrient in fruits and vegetables, has been found to reduce inflammation and improve metabolic function in old mice.

"Think of aging as being in an oven," DePinho says, "You're slowly getting cooked over time. But you can turn the heat down and let something cook longer."

5 habits for healthy aging

The stunning advances coming in human aging — perhaps shaping your old age — make the best case for preparing now by following lifestyle choices that research has proven will maximize your chance of a healthy old age — and even extend longevity.

"It's never too late," Verdin says. "Twenty years from now, an 80-year-old who had followed the best practices for good health — and once things are discovered and approved by FDA — is likely to be 10 years younger physiologically."

In 2015, the Finnish Geriatric Intervention Study to Prevent Cognitive Impairment and Disability (FINGER) trial demonstrated for the first time that a group of lifestyle interventions can improve brain health and prevent cognitive decline. Researchers followed 1,260 older adults (60 to 77 years old) over a two-year period to investigate the link between lifestyle choices and cognitive health. They found a 25 percent greater improvement in cognition among people who adhered to healthy lifestyle behaviors than among those in the control group who received general health advice.

The five "fingers" in the study are associated with cognitive health, and researchers generally agree on the lifestyle practices that maximize healthy aging:

Exercise. Studies have shown that 30 minutes of exercise a day can help control weight, boost energy, enhance cognitive function and promote better sleep. Even 15 minutes a day is useful. "If you exercise even a little, it will expand your health expectancy," says the NIA's Ferrucci.

Healthy eating. Although there is no single diet that is conclusively proven to slow aging, many experts recommend a Mediterranean plant-based diet that is rich in fruit and vegetables, whole grains, legumes, fish and nuts. Minimal intake of processed foods, sugar, alcohol and red meat is also recommended. The Mediterranean diet has been proven to be effective in reducing cardiovascular disease and risk factors such as high cholesterol, high blood pressure, glucose levels and inflammatory markers, all of which are associated with cognitive decline. A 2022 study published in The

American Journal of Clinical Nutrition found that a Mediterranean-style diet was associated with slower age-related brain atrophy.

Cognitive engagement. According to neuroscientist Rogalski, numerous kinds of mental activities can help maintain general cognitive function and brain health. These may include learning a new language, playing strategy games such as chess or checkers, or working through difficult puzzles. The key is to choose activities that are mentally challenging.

Social activity. Interact with friends and family in activities. It can boost your cognition and is emotionally fulfilling. A 2017 study by Rogalski's team, published in PLOS One, found that super agers who engaged in more social relations and relationships had better memory scores than older adults who did not engage.

Cardiovascular health. Stay on top of your blood pressure and cholesterol. Statins, which have been prescribed for decades, are among the most effective antiaging drugs already on the market. (They are also anti-inflammatory.) Medications to lower high blood pressure, which is a primary marker of health, are also widely available.

Be sure to have regular exams that include blood work, a colonoscopy, a mammogram, a prostate check and a routine diabetes check.

Talk to your doctor about supplements such as vitamin D, calcium and B12. Although the internet is abundant in supplements promising all sorts of miracles, don't take those unless you have discussed it with your doctor.

Regular dental appointments are important, too. A recent analysis led by the NIA found that the bacteria associated with periodontal disease are also associated with the development of Alzheimer's and related dementias, especially vascular dementia.

"If we intervene in the first part of life, we may not be able to eliminate disease, but maybe we can compress it closer to the end of life," Ferrucci says. "Most older people are not afraid of dying, they're more afraid of becoming disabled and dependent on others. If we could expand the good life, we can prolong longevity. The problem is that we're only intervening on disease when disease becomes clinically evident."

Geroscientists stress that these kinds of advancements in health span are more important than holding out hope that we'll eventually be able to live to 200. But living to 100, in excellent health, seems to be a commonly attainable goal in the not-too-distant-future.

In September 2000, two researchers made a friendly wager. Biologist Steven Austad predicted that someone alive that year would live to 150. Longevity researcher S. Jay Olshansky challenged him to a bet, arguing that no matter what advances were made in antiaging medicine, no human who was alive in 2000 would still be around in 2150.

Both men invested $150. They've since doubled the bet, and with returns, the pot could feasibly exceed $1 billion by 2150. The only question is, whose descendants will reap the rewards?

Olshansky, a professor in the School of Public Health at the University of Illinois at Chicago, remains a skeptic: "I'm more confident today that I will win this bet than when I first made it 24 years ago. There is no evidence to suggest anyone will live past 120, let alone 150."

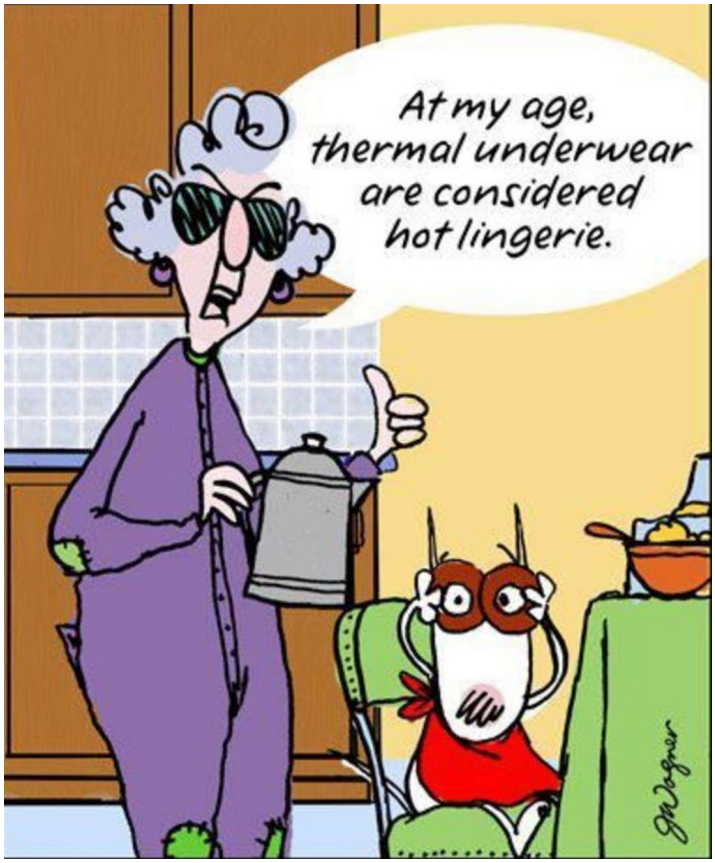
Austad, who is AFAR's scientific director, counters, "We have learned since the bet that life-extending drugs (at least in mice) have a substantial longevity boost even when begun as late as 60 years or more. I'm fairly confident that such drugs will be available within 20 years. But even if it takes 30 years, I'm confident that I'll win."

And that could add up to an awful lot of candles on our birthday cakes.

N	L	G	I	E	G	I	O	G	B	A	T	I	N
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I	E	I	G	F	E	E	O	S	S	A	L	O	T
T	R	Y	N	O	T	I	N	S	O	O	G	P	N
C	A	F	R	N	D	S	E	B	G	N	E	C	E
E	N	N	S	D	G	N	O	I	I	C	E	D	S
F	E	O	V	N	R	R	A	R	R	E	D	C	F
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S	I	C	I	N	N	G	S	R	N	I	R	H	I
B	O	Y	F	R	I	E	N	D	D	A	H	F	G



Laughter is the Best Medicine



COMMUNITY BULLETIN

NEW MEXICAN TRAIN DOMINO GAME

Come check out the new and exciting game of Mexican Train Domino! It's a game that's good for brain health, helps with problem solving and improve memory while having fun with your fellow players! First meeting is on Wed. Jan 8 at 1:30. Refer to the Activity Calendar for the meeting dates.

PINOCLE FUN

Looking for players who enjoy strategic trick-taking card games. Contact Patty at 650 787-6080 and join the fun.

ELDER CARE

If you are a senior looking for assistance with grocery shopping, caregiving, need a ride to appointments, errands, picking up medications, run to the post office, light housekeeping or other tasks, please contact Michael at 510-604-5646. I live in Mission Bay.



SEEKING CHINESE TRANSLATOR

The VIEU is seeking a new Chinese translator to assist with communicating with our Chinese residents by translating monthly safety updates and articles. Please contact Pete Cleaveland at 510-351-1742.

POKER PLAYERS NEEDED

Wednesdays 12 - 3:30 at the Mission Bay Clubhouse



Anyone with old, torn, or faded American flag, may drop them off at 239 Santa Susana for proper disposal.

NOTICE

Please contact me at missionbaynews@gmail.com to let me know if someone on the birthday list has

Senior Resource Guide and Important Phone Numbers

Alameda County Adult Protective Services	1 (866) 225-5277
Alameda County Crisis Support Services (24hr)	1 (800) 309-2131
Alameda County In-Home Supportive Services	1 (510) 577-1900
Alameda County Area Agency on Aging	1 (510) 577-3530
Alzheimer's Services of the East Bay	1 (510) 644-8292
American Diabetes Association	1 (510) 654-4499
Arthritis Foundation	1 (888) 391-9389
CA Dept. of Aging	1 (800) 510-2020
California Relay for Hearing Impaired	7-1-1
California Smoker's Helpline	1 (800) 662-8887
Center for Independent Living	1 (510) 841-4776
City of San Leandro Minor Home Repair	1 (510) 625-0316 x102
Consumer Fraud Reporting	1 (415) 553-7400
Covered California Hotline	1 (800) 300-1506
Davis Street Family Resource Center	1 (510) 347-4620
DMV	1 (800) 777-0133
ECHO Housing	1 (510) 581-9380
Emergency Services (Ambulance, Fire and Police)	9-1-1
Family Caregiver Alliance	1 (800) 445-8106
Flex Rides - City of San Leandro	1 (510) 577-7985
Health Insurance Counseling & Advocacy	1 (510) 839-0393
Legal Assistance for Seniors	1 (510) 832-3040
LIFE ElderCare	1 (510) 894-0370
Medi-Cal	1 (510) 670-6000
Mercy Brown Bag Program	1 (510) 534-8540 x369
Mobilehome Assistance Center (Complaints) HCD	(800) 952-8356
Mobilehome Residency Law Protection Program HCD	(800) 952-8356
Ombudsman Program	1 (510) 638-6878
PG&E Smarter Energy Line	1 (800) 933-9555
Paratransit – East Bay Paratransit (AC Transit)	1 (510) 287-5000
San Leandro Adult School	1 (510) 667-6287
San Leandro Marina Community Center	1 (510) 577-3462
San Leandro Senior Community Center	1(510) 577-3462
Senior Legal Hotline of Northern California	1 (800) 222-1753
Social Security Administration	1 (800) 772-1213
S.O.S./Meals on Wheels	1 (510) 582-1263
Spectrum Community Services, Inc.	1 (510) 881-0300
Spectrum Lunch Reservations - San Leandro SCC	1 (510) 825-9793
Supplemental Security Income (SSI)	1 (800) 772-1213
Sutter Care at Home	1 (800) 698-1273
The Friendship Line (Institute on Aging)	1 (800) 971-0016
Traffic and Transit	5-1-1
Veterans Assistance Benefits	1 (800) 827-1000